Cleveland Clinic

Anemia During Pregnancy

You may have anemia during pregnancy if a CBC shows that your red blood cells are low. Red blood cells carry oxygen through your body. A low count can make you feel fatigued, dizzy, cold and out of breath. In most cases of anemia during pregnancy, tweaks to your diet can put you on the right path.

What Is Anemia During Pregnancy?

Anemia during pregnancy is when you don't have enough red blood cells to carry oxygen throughout your body. It's common. It often happens because your body makes more blood during pregnancy. It needs extra iron and vitamins to keep up with the increase in blood volume. Without enough of these nutrients, your body can't make the red blood cells you and the fetus need.

Types of anemia during pregnancy

There are more than 400 types of anemia. These types are more common during pregnancy:

- **Iron-deficiency anemia:** This is the most common type of anemia in the U.S. It comes from a lack of iron.
- **Folate-deficiency anemia:** This type is from a lack of folic acid.
- **Vitamin B12 deficiency anemia:** This type is from a lack of vitamin B12.

Symptoms and Causes

Symptoms of anemia in pregnancy

You may not notice any symptoms of anemia at first. Over time, you may feel:

- Dizziness or weakness
- Extreme tiredness
- Headache
- Shortness of breath

Other symptoms include:

- Fast heartbeat
- Trouble concentrating
- Pale skin

Symptoms of anemia can feel a lot like normal pregnancy symptoms. That's why you'll still have blood tests during pregnancy to check for anemia.

Anemia during pregnancy causes

Pregnancy itself is a cause of anemia because your blood volume increases so much. You may also get it if you have a lack of iron, vitamin B12 or folic acid.

Other common causes of anemia can also cause anemia during pregnancy:

- Certain diseases, like sickle cell anemia and thalassemia
- Donating blood
- Heavy periods (before pregnancy)
- Ulcers and polyps

Risk factors

You may be at higher risk for anemia during pregnancy if you:

- Had anemia before pregnancy
- Are pregnant with multiples
- Don't eat enough foods with iron
- Have back-to-back pregnancies with little time between
- Had a heavy menstrual flow before pregnancy
- Vomit often because of morning sickness

Lowering your risk

The best thing you can do to lower your risk is eat at least 27 milligrams (three servings) of iron each day. If you can't get that much iron in your food, talk to your provider about taking an iron supplement. You should also take a prenatal vitamin every day.

Keep in mind that you can do all the right things and still get mild anemia during pregnancy. Contact your healthcare provider if you have symptoms of anemia.

Does anemia in pregnancy affect my baby?

Anemia can affect the growth and development of the fetus, especially during the first trimester. That's because it relies on you for oxygen, iron and nutrients.

Complications of anemia during pregnancy

Untreated anemia can get worse over time. Having too little oxygen in your blood can damage your organs. It also forces your heart to work harder, increasing your risk of:

- Irregular heartbeat
- Enlarged heart
- Heart failure

Severe anemia can cause complications for your baby, too. It raises your risk of preterm birth and having a baby born at a low birth weight. Your baby is also at higher risk of having anemia after birth.

Diagnosis and Tests

How doctors diagnose anemia

A blood test called a complete blood count (CBC) can diagnose anemia. Your provider usually orders this test at one of your first prenatal appointments.

Your healthcare provider uses the CBC to analyze:

- How many red blood cells you have
- The size and shape of your red blood cells
- How much iron your body has stored
- If you're low in vitamins B12 and B9

Management and Treatment

How is anemia in pregnancy treated?

Treatment for anemia depends on how serious it is. If you have:

- **Mild to moderate anemia:** Your provider will usually treat it with an iron supplement or by changing your prenatal vitamin.
- **Severe anemia:** You may need a blood transfusion. This will give you a healthy amount of red blood cells. This is rare.

You should begin to feel better after a few days of taking an iron supplement. If you don't notice a change, talk to your provider.

What foods should I eat for anemia?

Eat more foods that are high in iron, like spinach, lean red meat, chicken, turkey and eggs. Foods that are high in vitamins that help your body absorb iron (like vitamin C) are important, too. This includes oranges, strawberries, tomatoes and peppers.

When should I see my healthcare provider?

Talk to your provider if you experience signs of anemia, like:

- Dizziness and weakness
- Headaches
- Fast heartbeat
- Pale skin

If you're worried, ask your provider about testing for anemia and what you can do to maintain healthy red blood cell levels throughout your pregnancy.

Outlook / Prognosis

What can I expect if I have anemia during pregnancy?

Having anemia during pregnancy can be challenging. But it's treatable and not usually a cause for concern. Most women with anemia have mild forms. Taking an iron supplement, adjusting your prenatal vitamin and/or eating iron-rich foods is usually all you need. Your pregnancy care provider will monitor you with blood tests to make sure treatment is working.

The good news is that your blood volume will return to normal once your baby is born. You also won't have the same nutritional demands as you had during pregnancy. This usually means anemia improves or goes away after delivery.

A note from Cleveland Clinic

During pregnancy, your body makes a lot more blood — about 20% to 30% more! That can sometimes cause anemia. Eating iron-rich foods and taking vitamin C and B vitamins can help. Your prenatal vitamin helps, too. Talk to your provider if you have signs of anemia, like weakness, dizziness or irregular heartbeat. They can check your red blood cell count and recommend treatment.

Experts You Can Trust

Medically Reviewed. Last updated on 06/05/2026.

References

Cleveland Clinic's health articles are based on evidence-backed information and review by medical professionals to ensure accuracy, reliability and up-to-date clinical standards.

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